Healthcare App Idea

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what is a good app for a congressoinal app challange submission, that i...

Thinking ● Searched 10 websites ✓ Worked for a couple of seconds If...

needs to be a specific, tangible, painful real world problem that is being...

Thinking ● Searched 7 websites ✓ Worked for a couple of seconds Yes....

I think RecoverAI is the best one.

Thinking ● Searched 11 websites ✓ Worked for a couple of seconds I...

what is a good app for a congressoinal app challange submission, that is related to healthcare, machine learning, data science, computational neuroscience.

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Thinking

- **Searched 10 websites**

✓ Worked for a couple of seconds

If you're aiming for a **strong Congressional App Challenge submission** that genuinely combines **healthcare + machine learning + data science + computational neuroscience**, I would avoid making another generic "AI diagnoses disease from an image" app.

The 2025-2026 CAC results actually make that distinction important. Healthcare AI is already a fairly common winning-app category: examples include anemia detection, Parkinson's detection, pneumonia/chest-disease analysis, and patient communication. (House of Representatives) The CAC's official judging criteria emphasize **originality, implementation/UX, and demonstrated coding ability**, so the technical depth and uniqueness matter as much as the healthcare problem. (Congressional App Challenge)

Given your interests, I think there is a much stronger direction:

My top idea: NeuroTwin

NeuroTwin: An AI-powered computational model of the brain that helps visualize and predict how neurological disorders affect brain networks.

Instead of saying:

> *"Upload a medical image and AI tells you if you have a disease."*

you build an interactive **computational neuroscience simulator**.

The core idea

A user selects a neurological condition such as:

- Parkinson's disease
- Alzheimer's disease
- Epilepsy
- Stroke

The app constructs a simplified computational model of relevant brain regions and their connections.

For example, with Parkinson's:

Motor cortex → Striatum → GPe → STN → GPi → Thalamus → Motor cortex

The user can manipulate parameters such as:

- dopamine availability
- neuronal firing rates
- connectivity strength
- stimulation frequency
- stimulation amplitude
- disease severity

Then your ML model predicts how those changes affect the network.

You could visualize:

- neural activity over time
- firing-rate distributions
- network connectivity
- abnormal oscillations
- simulated motor/cognitive outcomes
- before vs. after treatment

That is much closer to actual **computational neuroscience** than simply putting an AI model behind a healthcare UI.

The really interesting part: treatment simulation

This is where I think your project could become substantially more impressive.

You could make the app a **"What if?" brain simulator.**

For example:

> *Parkinson's patient simulation*
> *Dopamine: 35%*
> *STN activity: High*
> *Beta-band oscillation: High*

Then the user can try:

Deep Brain Stimulation

Frequency:
`130 Hz`

Amplitude:
`2.0 mA`

Pulse width:
`60 µs`

The simulation predicts changes in the network.

Then you could compare:

No treatment

Motor network instability: **High**

DBS configuration A

Motor network instability: **Medium**

DBS configuration B

Motor network instability: **Low**

And potentially introduce a second objective:

Motor improvement vs. cognitive side effects

That would be extremely interesting because it creates a genuine computational neuroscience problem rather than a simple classifier.

Even better: personalized "digital twin"

You could take the concept one step further.

The user enters simulated/patient parameters:

```
Age
Disease duration
Motor symptoms
Cognitive symptoms
Dopamine level
Neural activity
Medication
DBS parameters
```

Your model then generates a simplified **personalized brain network**.

Something like:

```
            MOTOR CORTEX
                 |
                 ↓
              STRIATUM
                 |
        ↓ dopamine
                 |
               STN
             ↙     ↘
      GPi            GPe
        \             /
         \           /
          THALAMUS
              |
              ↓
        MOTOR CORTEX
```

The app could show the network dynamically changing as the disease progresses or treatment parameters change.

This gives you a very strong phrase for the project:

> ***"A computational digital twin for exploring neurological disease and treatment."***

Where machine learning comes in

You don't want the ML to just be a decorative chatbot.

I'd give it a real job.

For example, train an ML model to predict:

Input

```
dopamine
STN firing rate
GPi firing rate
connectivity
DBS frequency
DBS amplitude
disease severity
```

Output

```
motor symptom score
cognitive impact score
network stability
treatment response
```

You could train it using simulated data generated from your computational model.

That gives you an interesting architecture:

Computational neuroscience model

↓

Generate thousands/millions of simulated brain states

↓

Machine-learning model

↓

Learn relationship between network parameters and outcomes

↓

Interactive app

↓

User changes parameters

↓

ML predicts outcome

That is much more sophisticated than simply calling an API.

A second idea I really like: NeuroLens

Another direction would be:

NeuroLens: AI analysis of neurological signals

The user uploads or records a signal such as:

- EEG
- tremor data
- accelerometer data
- voice
- reaction time
- tapping/movement data

Your app extracts features using data science:

- frequency-domain features
- spectral power
- entropy
- variance
- signal complexity

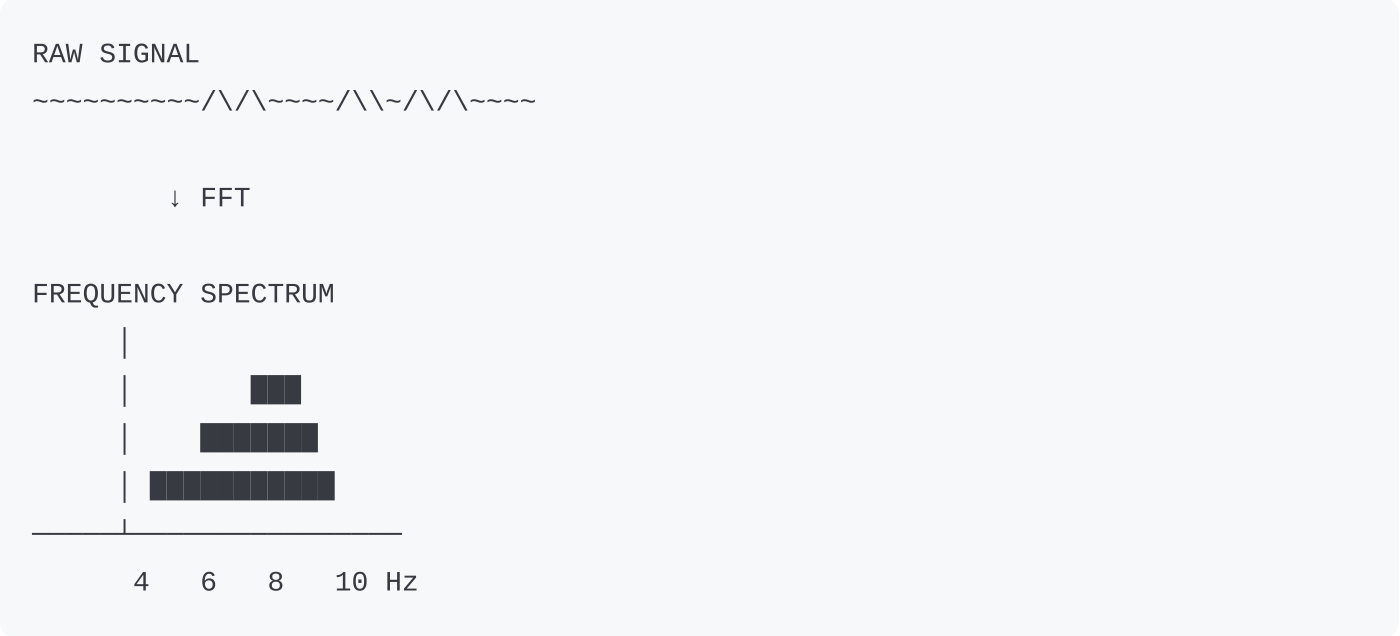
- movement frequency
- tremor amplitude

Then an ML model analyzes the signal.

For Parkinson's, for example:

Phone accelerometer → tremor signal → signal processing → ML → tremor characteristics

You could visualize the raw signal and transformed signal:



And then provide:

> *"Your movement pattern contains characteristics associated with tremor."*

Importantly, I'd frame it as **research/educational screening**, not a medical diagnosis.

This would let you demonstrate much more actual data science.

Third idea: BrainGraph

This one would lean even harder into computational neuroscience.

BrainGraph: Explore how neurological diseases change brain networks

Build an interactive 3D/2D brain network.

Each node represents a brain region.

Edges represent functional/structural connections.

The user can select:

Healthy

vs.

Parkinson's

vs.

Alzheimer's

vs.

Stroke

Then the app visualizes changes in:

- connectivity
- centrality
- network efficiency
- clustering
- modularity
- information flow

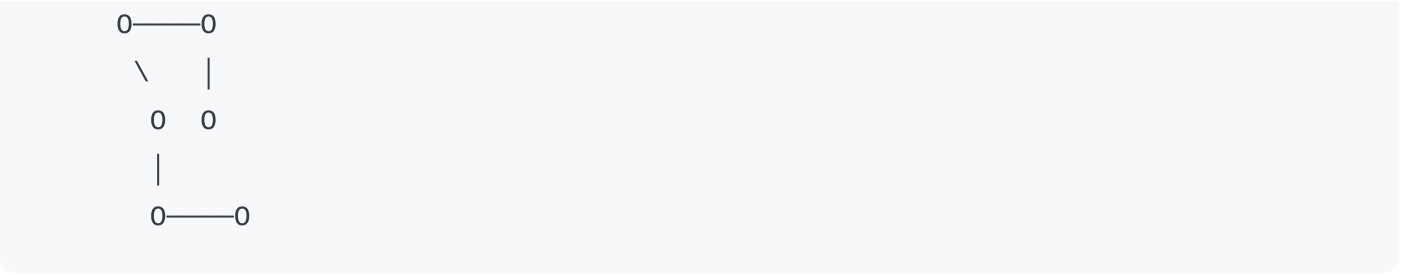
You could calculate graph-theoretic metrics using Python.

For example:

```
Healthy Brain

     0———0
    /|    |\
   0–0———0–0
    \|    |/
     0———0


Parkinson's
```



Then ML could predict disease state from the network's features.

This would be a **very legitimate computational neuroscience/data science project**.

Fourth idea: NeuroRehab AI

This would be more directly user-facing.

NeuroRehab AI

A webcam/phone camera analyzes rehabilitation exercises.

For example:

Parkinson's rehabilitation

The user performs:

- finger tapping
- hand opening/closing
- arm movement
- walking
- balance exercises

Computer vision extracts:

- movement speed
- range of motion
- tremor
- hesitation
- asymmetry
- movement consistency

Then the app creates a longitudinal dataset.

Instead of simply:

> *"You scored 72."*

you show:

```
WEEK 1
Movement speed     61
Consistency        54
Tremor             72

WEEK 4
Movement speed     68 ↑
Consistency        71 ↑
Tremor             59 ↓
```

That creates a compelling **personal health data science** component.

Fifth idea: BrainDose

This one is probably the most ambitious.

BrainDose: Optimize neurological treatment using computational models

You could create a simplified computational model of DBS.

The user chooses a simulated patient.

Your system searches through thousands of possible DBS parameter combinations.

For example:

```
Frequency:     60-180 Hz
Amplitude:     0.5-4.0 mA
Pulse width:   30-120 µs
```

Then your algorithm searches for a configuration that maximizes:

motor improvement

while minimizing:

cognitive/network disruption

You could use:

- optimization algorithms
- neural networks
- reinforcement learning
- Bayesian optimization
- genetic algorithms

This is particularly aligned with the work you've been doing around **basal ganglia + DBS**.

What I'd actually choose for you

If this were **your 2026 CAC project**, I'd rank them:

Idea	Technical depth	Healthcare	Neuroscience	Originality	CAC potential
NeuroTwin	10/10	9/10	10/10	9/10	**10/10**
BrainDose	10/10	9/10	10/10	10/10	**10/10**
NeuroLens	9/10	9/10	8/10	8/10	9/10
BrainGraph	9/10	8/10	10/10	9/10	9/10
NeuroRehab AI	8/10	10/10	8/10	7/10	9/10

But I would combine #1 and #2.

I'd make the actual project:

NeuroTwin

An AI-powered computational digital twin for neurological disease and treatment optimization

The app would have **four major components**:

1. Brain Model

Interactive basal-ganglia/neural-network simulation.

2. Disease Simulator

Change disease severity and observe how neural dynamics change.

3. Treatment Simulator

Experiment with DBS parameters.

4. AI Optimizer

ML/optimization model searches for treatment parameters that balance:

motor improvement + network stability + cognitive preservation

That last part is what could make the project stand out.

Why I think this is better for you specifically

You already have experience with **computational neuroscience, Parkinson's, DBS, Python, neural networks, and healthcare AI**, so you could build something that is substantially deeper than a generic high-school medical AI app.

And there is precedent that neuroscience/neurological-health apps can do very well at CAC. In 2025, Massachusetts' 2nd District selected **NeuroLens**, while California's 10th District selected **Synapse Match**, an AI Parkinson's screening application. (House of Representatives)

The catch is that **another Parkinson's detection app would now feel comparatively familiar**. Your advantage would be moving from:

> ***"Can AI detect Parkinson's?"***

to:

> ***"Can we computationally model what is happening inside the diseased brain and use that model to explore how treatment changes the system?"***

That is a much more distinctive CAC story.

Also, CAC explicitly evaluates **creativity/originality, implementation, UX/design, and coding/programming excellence**, which makes a project like this particularly attractive because you can demonstrate all four rather than relying solely on an impressive medical claim. (Congressional App Challenge)

If you want, I can also come up with **10 completely different CAC ideas** in this same healthcare + ML + computational neuroscience space, including some that are **much more likely to be buildable by October 2027** and some that are **extremely ambitious/award-oriented**.

Sources

Congressional App Challenge | house.gov house.gov

2026 CAC Rules congressionalappchallenge.us

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needs to be a specific, tangible, painful real world problem that is being solved here, not some theoretical "research purposes" app.

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- **Searched 7 websites**

✓ Worked for a couple of seconds

Yes. That changes the direction substantially.

You need to start with a **specific failure in healthcare**, not with "here is a cool ML/neuroscience technology." The ML/computational neuroscience should be the mechanism that makes your solution possible.

And I think there is one problem that fits your interests extremely well:

Best idea: an AI home-rehabilitation monitor for stroke survivors

The real-world problem

A stroke survivor leaves the hospital and is prescribed rehabilitation exercises.

The problem is that **the clinician usually isn't there when the patient actually performs them at home**.

That creates several concrete failures:

- The patient may perform an exercise incorrectly.
- They may do fewer repetitions than prescribed.
- Their affected arm may gradually worsen without anyone noticing.
- They may stop doing therapy altogether.

- The physical therapist may not know what happened between appointments.
- Traveling to frequent therapy appointments can be difficult or expensive.

This isn't theoretical. A 2026 study found that poor adherence to home rehabilitation remains a persistent problem for stroke survivors, while geographic barriers, travel, insurance restrictions, and time make conventional follow-up difficult. (PubMed Central (PMC))

And there is an especially interesting ML opportunity: a 2026 study used the first three weeks of home-rehabilitation behavior to predict whether patients would drop out in week four, achieving an AUC of 0.810. (PubMed)

So you have a **specific, measurable failure**:

> ***"Stroke patients are expected to rehabilitate at home, but clinicians cannot continuously see whether patients are doing the exercises correctly or whether they are beginning to disengage."***

That's a real problem.

The app: RecoverAI

A patient opens the app on their phone/tablet and sees:

> ***Today's rehabilitation***
>
> *3 exercises*
> *10 repetitions each*
> *~12 minutes*

They prop their phone up.

The camera watches them perform the exercise.

Example: affected-arm rehabilitation

The patient is told:

> *Raise your affected arm to shoulder height 10 times.*

Computer vision tracks their body landmarks.

Instead of merely counting:

10/10 repetitions

your model analyzes the movement:

- range of motion
- movement speed
- joint angle
- smoothness
- asymmetry between arms
- compensatory movements
- repetition consistency

Then:

> ***8/10 repetitions completed correctly***
>
> *Your affected shoulder reached 74° on average.*
>
> *Your previous average: 81°*
>
> ***Range of motion decreased 9%.***

That is immediately useful.

But here's the part that makes it CAC-worthy

Don't make it just an exercise counter.

Build a **clinician-facing longitudinal dataset**.

The app tracks:

```
Day 1
ROM: 81°
Speed: 0.74 m/s
Consistency: 86%
Repetitions: 10/10
```

```
Day 7
ROM: 78°
Speed: 0.71 m/s
Consistency: 83%
Repetitions: 9/10

Day 14
ROM: 70°
Speed: 0.63 m/s
Consistency: 71%
Repetitions: 7/10
```

Your ML system recognizes:

> ***The patient's rehabilitation performance has been deteriorating over the past two weeks.***

And instead of waiting until their next appointment, the app can say:

> ***Your recent movement performance has changed significantly. Consider contacting your physical therapist.***

The clinician dashboard could show:

Patient #104

Recovery trend: declining

Last 7 days

- Exercise adherence: 62%
- Average ROM: ↓ 13%
- Movement consistency: ↓ 18%
- Affected-arm usage: ↓ 11%

That is a **real intervention**.

The neuroscience component isn't forced

This is where you can differentiate yourself from the dozens of generic "AI physical therapy" projects.

Stroke rehabilitation is fundamentally about the nervous system's ability to **reorganize and recover function**.

Your app could model the patient's recovery as a time-series of motor-function measurements.

You could calculate things like:

- movement variability
- motor asymmetry
- reaction time
- coordination
- movement smoothness
- frequency-domain characteristics
- progression over time

Then use ML to identify **early deviations from the patient's own recovery trajectory**.

The key is:

Don't ask:

> *"Does this person have a neurological disease?"*

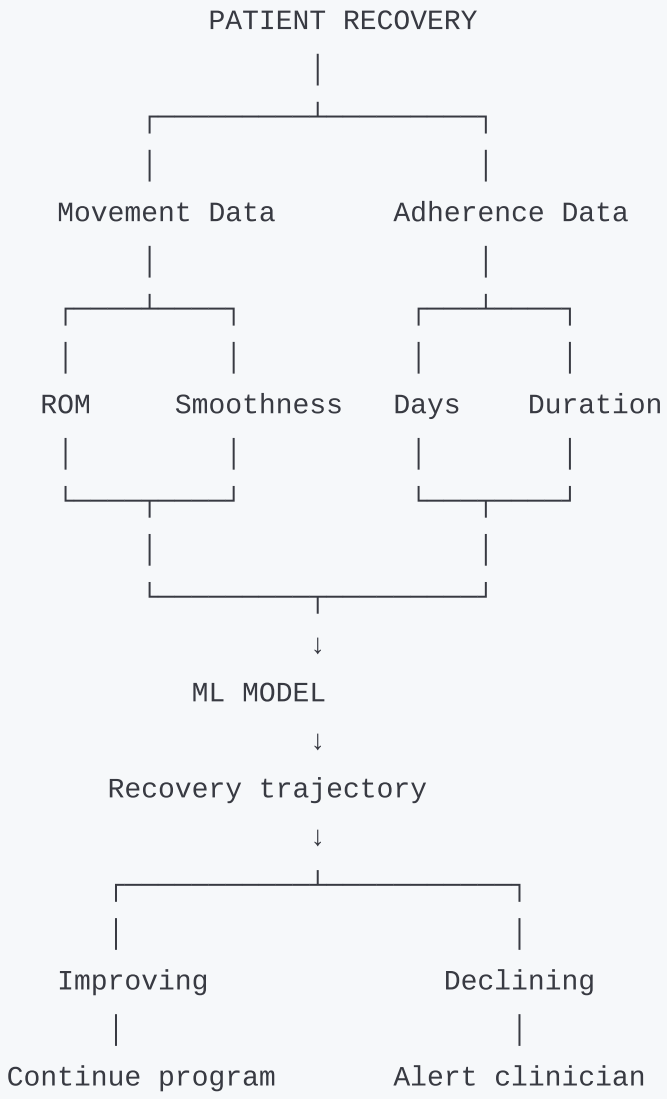
Ask:

> ***"Is this person's recovery trajectory changing in a way that warrants intervention?"***

That's a much more practical problem.

And you can make it surprisingly sophisticated

Imagine the app sees:



Now your ML actually has a **job**.

There is evidence this is technically feasible

This isn't something you're inventing out of thin air.

Researchers have already demonstrated ML systems that classify rehabilitation movements using inexpensive wearable sensors, specifically because conventional home rehabilitation has problems with monitoring, feedback, cost, and adherence. (Nature)

There are also recent systems using computer vision and sensors to assess stroke rehabilitation movements at home. (ScienceDirect)

And importantly, remote therapeutic monitoring has been shown to improve home-exercise completion in stroke rehabilitation. A 2026 study found that remote

monitoring increased exercise completion by about **3.3 hours over three weeks**. (PubMed Central (PMC))

So your project isn't:

> *"I wonder if AI could help stroke patients."*

It's:

> ***"There's a documented gap in post-stroke rehabilitation monitoring. I'm building a low-cost system that uses computer vision and machine learning to quantify what happens between physical-therapy appointments."***

That's a considerably stronger CAC pitch.

Another idea I'd seriously consider: Parkinson's medication + symptom monitoring

This could potentially fit you even better because of your computational neuroscience work.

Problem

Parkinson's patients can experience substantial fluctuations in movement throughout the day.

A neurologist may see the patient for **15-30 minutes**, while the patient's actual symptoms occur over many hours at home.

So the clinician might not get a good picture of:

- tremor
- bradykinesia
- gait changes
- medication response
- "on/off" periods

App

The patient periodically performs a **30-second standardized motor test** on their phone.

For example:

Finger tapping

Left:

`• • • • • • •`

Right:

`• • • •`

Your model measures:

- tapping frequency
- amplitude
- hesitation
- variability
- asymmetry

The patient could also record a short walking segment.

Over weeks:

```
08:00 AM
Medication taken

08:30 AM
Tapping speed: 4.8/sec

11:30 AM
Tapping speed: 3.9/sec

02:30 PM
Tapping speed: 2.7/sec
```

The app could identify a recurring relationship between medication timing and motor performance.

Then:

> ***"Your movement performance has consistently declined approximately 4 hours after your recorded medication time. Share this report with your neurologist."***

That's a **real clinical communication problem**.

And it naturally combines:

computational neuroscience + time-series analysis + ML + healthcare + mobile computing.

Another genuinely painful problem: falls

This one is even more tangible.

Problem

Older adults and people with neurological disorders can fall at home.

The problem isn't simply detecting a fall.

It's:

> ***What happens in the minutes after the fall?***

Someone could be:

- unable to stand
- disoriented
- injured
- unconscious
- unable to reach their phone

A phone/watch-based app could use accelerometer + gyroscope data to detect a probable fall.

But then your ML model could distinguish:

normal sitting down

from

rapid fall

from

collapse

and ask:

> ***"Are you okay?"***

If there is no response, it could escalate to a designated caregiver.

That is an extremely tangible problem.

However, I'd be careful with this one because reliable emergency detection has a much higher safety bar.

What I would NOT build

I agree with your criticism of my previous answer.

I would eliminate:

"Explore your brain"

"Interactive brain simulator"

"Learn about Parkinson's"

"Digital twin for research"

"What happens to the brain when..."

unless they're attached to an actual patient problem.

Those can be excellent research projects.

They're weaker **Congressional App Challenge products** because a judge can ask:

> *"Who actually needs this?"*

and the answer becomes vague.

My top 4 for you

Project	Actual problem	ML	Neuroscience	Tangible user	CAC potential
RecoverAI	Stroke patients aren't adequately monitored between PT visits	10/10	9/10	Patient + PT	**10/10**
Parkinson's Daily Monitor	Neurologists don't see patients' symptoms throughout the day	10/10	10/10	Parkinson's patient	**10/10**
FallGuard	Falls can go unnoticed when someone is alone	9/10	8/10	Elderly/neurological patients	**9/10**
NeuroRehab Coach	Patients perform rehab incorrectly/ineffectively at home	9/10	9/10	Rehab patient	**9/10**

If I were choosing for you:

#1: Parkinson's Daily Monitor

because it fits your existing computational-neuroscience background exceptionally well.

But I would **not** make it a "Parkinson's detector."

I'd make it a **between-appointments monitoring system**.

The fundamental problem becomes:

> ***A neurologist gets a snapshot of a Parkinson's patient's condition during an appointment. The patient's symptoms actually fluctuate throughout the day and between visits. This app turns short daily motor tests into a longitudinal dataset that can reveal changes and medication-response patterns the clinician would otherwise miss.***

That's specific.

That's painful.

That's measurable.

And, crucially, you can build a real working prototype without pretending that a high-school app can diagnose Parkinson's.

Sources

Remote Therapeutic Monitoring Enhances Adherence to a Home Exercise Program in Chronic Stroke - PMC pmc.ncbi.nlm.nih.gov

Forecasting Dropout in Home-Based Movement Rehabilitation After Stroke With Sensors and Machine Learning - PubMed pubmed.ncbi.nlm.nih.gov

Empowering stroke recovery with upper limb rehabilitation monitoring using TinyML based heterogeneous classifiers | Scientific Reports nature.com

Evaluating generalization of arm movement identification using machine learning: From structured to semi-structured environments - ScienceDirect sciencedirect.com

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I think RecoverAI is the best one.

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Thinking

- **Searched 11 websites**

✓ Worked for a couple of seconds

I agree. **RecoverAI is probably the strongest direction of the ideas we've discussed**, especially for a Congressional App Challenge project.

And after checking the current literature, I think we can sharpen it considerably. There is very recent evidence that the underlying problem is real: a 2026 randomized-trial analysis found that remote therapeutic monitoring increased home-exercise completion among chronic stroke survivors by **3.3 hours over three weeks**. Another 2026 study found that the first three weeks of home-rehabilitation behavior could predict who would drop out during week four, with an ML model achieving an AUC of **0.810**. (PubMed Central (PMC))

So I would define RecoverAI like this:

RecoverAI

AI-powered home rehabilitation monitoring for stroke survivors

The problem:

> *After a stroke, patients are often sent home with rehabilitation exercises, but clinicians cannot see what happens between appointments. Patients may perform exercises incorrectly, stop doing them, or gradually lose function without their therapist knowing.*

That gap is the product.

The actual app

1. Patient opens RecoverAI

The app says:

> ***Today's rehabilitation***
>
> *Arm Raise - 10 reps*
> *Elbow Flexion - 10 reps*
> *Reach & Place - 10 reps*

The patient puts their phone/tablet on a table.

No specialized equipment.

2. Computer vision watches the exercise

Using the camera, RecoverAI extracts body landmarks.

For each repetition, it measures things like:

- joint angles
- range of motion
- movement speed
- acceleration
- smoothness
- asymmetry
- compensatory movements
- repetition quality

So instead of:

> *"You completed 10 reps."*

it can say:

> ***8/10 repetitions completed correctly.***

> *Your affected arm reached an average of 74°.*
>
> *Your target is 90°.*
>
> *Your range of motion has decreased 8% compared with your previous sessions.*

That is tangible.

---

3. The important part: longitudinal monitoring

This is what I would make the centerpiece of the app.

Imagine a patient's therapist opens their dashboard:

Patient: John

Metric	Week 1	Week 2	Week 3
Exercise completion	91%	84%	67%
Avg. ROM	82°	79°	71°
Movement smoothness	76	72	63
Correct repetitions	88%	81%	69%

RecoverAI detects a pattern:

> ***Recovery performance has declined significantly over the past 14 days.***

The therapist can now investigate instead of discovering the problem weeks later.

That's a much stronger use of ML than simply classifying an exercise.

---

4. ML should predict a specific problem

I'd actually give RecoverAI **two ML models**.

Model A: Movement-quality model

Input:

```
pose sequence
joint angles
velocity
acceleration
symmetry
```

Output:

```
correct repetition
incorrect repetition
compensatory movement
```

This can be a temporal neural network, LSTM, 1D CNN, or transformer depending on how ambitious you want to get.

Model B: Rehabilitation-risk model

Input:

```
exercise frequency
session duration
repetition quality
ROM
movement speed
consistency
7-day trends
```

Output:

```
stable recovery
improving
possible decline
high dropout risk
```

This is particularly defensible because recent research has already demonstrated that behavioral signals from home rehabilitation contain predictive information about dropout. (Zendy)

And there's an even better feature

I'd add:

"Show my therapist"

The patient doesn't need to understand the ML.

They press:

Share Recovery Report

RecoverAI generates something like:

> ***Past 14 days***
>
> *Exercise adherence: 73%*
>
> *Average shoulder ROM: 76°*
>
> *Change from previous period: -11%*
>
> *Correct repetitions: 78%*
>
> *Movement consistency: -9%*
>
> ***Observation:** Declining performance detected across three consecutive sessions.*

The patient can send that report to their physical therapist.

That directly addresses the information gap between appointments.

This also gives you a very strong CAC story

You can demonstrate the entire pipeline to a judge in about two minutes:

Problem

A stroke survivor is sent home with exercises.

↓

Patient performs exercise

↓

Phone camera observes movement

↓

Computer vision extracts movement data

↓

ML evaluates exercise quality

↓

Data accumulates over days/weeks

↓

ML detects a concerning recovery trend

↓

Patient/therapist receives actionable report

That's an actual product.

One thing I would change from my original proposal

I would **not** market RecoverAI as:

> *"AI physical therapy."*

That's too broad, and there are already numerous AI-assisted rehabilitation systems. Recent reviews specifically identify computer-vision rehabilitation feedback and wearable-sensor systems as active areas of research. (Frontiers)

Instead:

> ***RecoverAI is a low-cost monitoring system that gives stroke survivors and their therapists objective information about what is happening during rehabilitation at home.***

That's much more specific.

And you have a compelling accessibility argument: existing research has explored smartphone-based home rehabilitation precisely because specialized clinical equipment can be expensive or impractical for home use. (ScienceDirect)

The feature set I'd target

MVP

- Account/patient profile

- Assigned exercises
- Camera-based pose tracking
- Automatic repetition counting
- Exercise-quality scoring
- ROM measurement
- Session history
- Progress graphs
- Therapist report

ML layer

- Exercise classification
- Incorrect-movement detection
- Personalized baseline
- Recovery-trend detection
- Dropout-risk prediction

Stretch goal

- Therapist dashboard
- Personalized exercise difficulty
- Voice feedback
- Multiple stroke-related exercises
- Offline/on-device inference
- Privacy-preserving processing

And I would make **on-device processing** a major selling point. A patient shouldn't have to upload video of themselves exercising to some random cloud server just to count arm movements.

The most important distinction

RecoverAI isn't trying to replace a physical therapist.

It's solving this:

> ***A therapist can observe perhaps an hour of a patient's rehabilitation every few weeks. RecoverAI helps capture objective information from the other hours and days when the patient is recovering at home.***

That is the core idea I'd build the entire CAC submission around.

Sources

Remote Therapeutic Monitoring Enhances Adherence to a Home Exercise Program in Chronic Stroke - PMC pmc.ncbi.nlm.nih.gov

Forecasting Dropout in Home-Based Movement Rehabilitation after Stroke with Sensors and Machine Learning | Zendy zendy.io

Frontiers | A narrative review of AI-driven stroke rehabilitation systems through the lens of human motor learning frontiersin.org

mRehab: A Community-engaging Study of an Ai-powered Mobile Rehabilitation System - ScienceDirect sciencedirect.com

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